

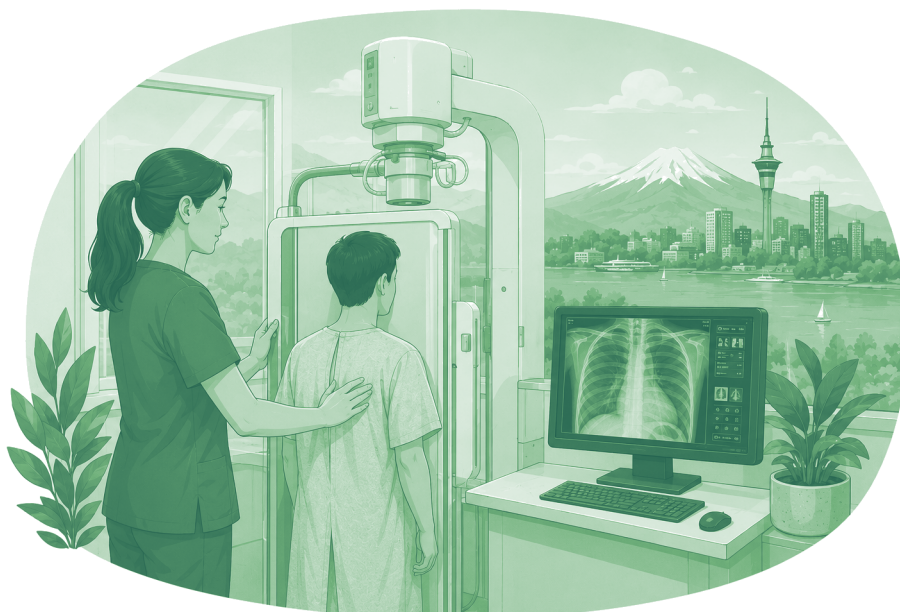


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Medical Radiation Technologists Board Registration Guide

Your complete guide to professional registration in New Zealand — explained in plain English, for internationally qualified medical imaging and radiation therapy professionals.



Welcome to your registration guide

For most internationally qualified professionals, registration with the Medical Radiation Technologists Board is the very first step on the road to working in New Zealand — and it usually needs to happen **before** you can apply for jobs.

That single fact surprises a lot of people. In many countries you find a job first and sort out the paperwork afterwards. New Zealand works the other way around: an employer generally wants to see that you are registrable — or already registered — before they progress your application. Understanding that order early changes how you plan everything that follows.

This guide explains the process from beginning to end. It is built from practical experience supporting internationally qualified healthcare professionals through registration, and it focuses on the things that genuinely make a difference: choosing the right referees, assembling clean documentation, and avoiding the small mistakes that quietly add weeks to an application.

How to use this guide

Read it once from start to finish to understand the shape of the journey, then keep it nearby and return to each section as you reach it. Think of it as a handbook rather than a leaflet — you'll get the most value by referring back to it throughout your application.

An important note

This guide is designed to **complement, not replace**, the official guidance published by the Medical Radiation Technologists Board. The Board sets the rules and makes all registration decisions. Where there is ever any difference between this guide and the Board's own published requirements, the Board's guidance always takes precedence. Ethicare supports and guides you through the process; we do not make registration decisions on the Board's behalf.

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A quick word before you start

Registration can feel daunting from the outside, but it is a structured, predictable process. Almost every delay we see comes down to a handful of avoidable issues — and this guide is organised specifically to help you sidestep them.

Understanding the MRTB

The Medical Radiation Technologists Board (MRTB) is the regulator responsible for the registration and oversight of medical radiation professionals in New Zealand.

The Board operates under the Health Practitioners Competence Assurance Act, the legislation that governs regulated health professions across New Zealand. Its core purpose is straightforward but important: **to protect the public** by making sure that everyone practising in a regulated imaging or radiation therapy role is suitably qualified, competent and fit to do so.

Why registration is required

In New Zealand, the titles and clinical roles within medical radiation are **legally protected**. You cannot lawfully practise — or call yourself a registered practitioner — without being registered with the Board and holding a current practising certificate. This is not a bureaucratic formality; it is the mechanism that assures patients, employers and colleagues that you have met a recognised national standard.

Top tip

Because registration generally comes before employment, treat it as the foundation of your whole relocation plan. Everything else — job offers, visa, relocation — sits on top of it.

Protected scopes of practice

The Board registers practitioners into defined **scopes of practice**. A scope describes the area of work you are qualified and authorised to perform — for example diagnostic imaging, sonography or radiation therapy. You are registered within a specific scope, and you must practise within it. Your qualifications and experience are assessed against the scope you are applying for, which is why matching your application to the correct scope from the outset matters so much.

The Annual Practising Certificate (APC)

Registration confirms that you have met the standard to practise. The **Annual Practising Certificate** is what then permits you to actually work, year to year. Think of registration as your licence and the APC as keeping that licence current. You must hold a valid APC at all times while working, and it is renewed annually — usually alongside a declaration about your health, conduct and continuing professional development.

Professional responsibilities & CPD

Being registered is an ongoing commitment, not a one-off achievement. Registered practitioners are expected to maintain their competence, practise ethically, and complete **Continuing Professional Development (CPD)** each year. CPD keeps your skills current and is part of what you confirm when you renew your APC. The Board can ask for evidence of it, so it is worth keeping good records from day one.



Which professions require registration?

The MRTB registers professionals across several distinct scopes of practice. If you work in any of the areas below, you will normally need to be registered before you can practise in New Zealand.

Medical Imaging Technologist

Also known internationally as a diagnostic radiographer, this is the largest group the Board registers. Medical Imaging Technologists produce diagnostic images using modalities such as general X-ray, CT, MRI, fluoroscopy and theatre imaging. The role combines technical precision with direct patient care, often across a wide range of clinical settings.

Sonographer

Sonographers use ultrasound to produce diagnostic images and information across obstetric, abdominal, vascular, musculoskeletal and other applications. The role is highly autonomous and interpretive, requiring strong clinical judgement alongside technical skill.

Mammographer

Mammography is a specialised area of breast imaging, often within national screening programmes as well as diagnostic and symptomatic services, and it calls for technical accuracy alongside a sensitive, reassuring approach with patients. It is worth understanding that mammography is **not a separate registration scope**: it sits *within* the Medical Imaging Technologist scope as an area you practise into with appropriate training. In practice that means you register as a Medical Imaging Technologist and develop mammography as a specialism, rather than applying for a standalone "mammographer" registration.

Nuclear Medicine Technologist

Nuclear Medicine Technologists work with radiopharmaceuticals to produce functional imaging — for example PET and SPECT studies. The role blends imaging expertise with the safe handling of radioactive materials and close attention to radiation protection.

Radiation Therapist

Radiation Therapists plan and deliver radiation treatment to patients, most often within oncology services. The role sits at the intersection of advanced technology and ongoing, compassionate patient care throughout a course of treatment.

Not sure which scope applies to you?

Your scope is determined by your qualification and the work you are trained and experienced to perform — not simply by your job title, which can vary from country to country. If you are unsure which scope your background maps to, Ethicare can help you identify the right one before you apply, so your application is assessed against the correct standard from the start.

Registration pathways

How your application is assessed depends largely on where and how you qualified. Understanding the likely pathway early helps you prepare the right evidence from the outset.

At its heart, registration asks two questions: **are you suitably qualified**, and **are you competent and safe to practise** within the scope you are applying for? The pathway your application takes is simply the route the Board uses to answer those two questions for your particular background.

How applications are assessed

Most international applications move through the same broad stages. Some applicants pass through smoothly; others are asked for additional information along the way. Neither outcome is unusual.

Qualification assessment

The Board first considers whether your qualification is comparable to the New Zealand standard for the scope you are seeking. Qualifications from some countries and institutions are well recognised; others may require closer examination or supporting evidence about course content and clinical hours.

Competence assessment

Beyond the paper qualification, the Board wants assurance that you are **currently** competent. This is where recent practice, employment references and your clinical history come into play. The aim is to confirm that your skills are current and that you can practise safely in a New Zealand setting.

Requests for additional information

It is common — not a red flag — for the Board to ask for further information during assessment. This might be a clearer document, an additional reference, or clarification of your scope or experience. Responding promptly and completely is the single biggest thing within your control to keep an application moving.

Possible outcomes

- **Registration granted** — you meet the standard and can proceed to obtain your APC.
- **Registration with a period of supervised practice** — a very common outcome for international applicants. You are registered and can work, but with a requirement to practise under the supervision of an experienced colleague for an initial period.
- **Conditions or further requirements** — registration may be granted subject to certain conditions, or pending additional evidence, assessment, or an examination.
- **Further assessment required** — the Board may request more information before a decision can be made.

A word about supervision

Many internationally qualified applicants are registered with an initial **period of supervised practice** — and it's important to understand that this is **normal, not a setback**. It is not a sign that the Board doubts your ability; it's a structured way of supporting you while you settle into the New Zealand health system, equipment, protocols and ways of working.

Whether supervision applies, and for how long, depends largely on how closely your training and regulatory background match New Zealand's. Applicants who trained and practised in countries with very comparable systems — such as the UK, Ireland, Australia and Canada — often face little or no supervised period. Applicants from other backgrounds are more commonly asked to complete one, and in some cases an examination may also form part of the pathway. The Board decides this case by case as part of its assessment, so it's worth expecting the possibility rather than being surprised by it.

Top tip

If a supervised period applies to you, factor it into your plans early — it affects the kind of role you'll start in, since you'll need an employer and a supervisor who can support it. Many employers are very used to this and will have supervision arrangements ready.

The online examination pathway

For some applicants, the Board's assessment leads to an **online examination** as the route to registration. The trigger is equivalence: the exam is offered where the Board reviews your application and concludes your qualification is **not straightforwardly equivalent** to the required New Zealand standard for your scope. It is a recognised, formal pathway — not a penalty.

This is where your scope matters. For Medical Imaging Technologists, Radiation Therapists and Nuclear Medicine Technologists, applicants from closely aligned systems (Australia in particular) often have a more streamlined route. **Sonographers and MRI Technologists do not have that shortcut**: even Australian-qualified applicants in these two scopes must apply through the full overseas-trained assessment, and there is no fast-tracked country pathway for Sonography at all. In practice that makes a full equivalence assessment — and therefore the examination route — **more likely if you're a Sonographer or MRI Technologist** than for some other scopes. It's worth anticipating from the outset rather than being caught out.

The exam is sat online under remote proctoring: a supervisor verifies your identity (you'll need two forms of ID, one government-issued photo ID, plus a unique authorisation code) and oversees the session. One practical point worth knowing — the examination fee is paid in advance, and it is forfeited if you're refused access on the day for failing to properly establish your identity, so have your documents in order.

A simple way to picture the assessment

1

Application received

The Board confirms your application and begins its review.

2

Qualification assessed

Your qualification is checked against the New Zealand standard for your scope.

3

Competence & recent practice assessed

Your references and recent practice confirm your skills are current.

?

Further information requested IF NEEDED

A routine step — the Board may ask for a clearer document or extra detail.

✓

Registration decision

The Board confirms your registration, sometimes with conditions.

Eligibility requirements

The Board's requirements can look like a long checklist. They are far easier to satisfy once you understand **why** each one exists — so that's how we'll explain them.

Qualifications

What is required: a recognised qualification appropriate to the scope you are applying for.

Why it exists: your qualification is the foundation of safe practice. The Board needs confidence that your training covered the theory and clinical competencies expected of a practitioner in New Zealand — which is why course content and clinical hours sometimes matter as much as the certificate itself.

As a guide to the level expected, the New Zealand qualifications that lead to registration are bachelor's degrees — for example the Bachelor of Health Science (Medical Imaging) or the Bachelor of Radiation Therapy. On the New Zealand Qualifications Framework, a bachelor's degree sits at **Level 7**, so that is broadly the benchmark your overseas qualification is compared against. The Board doesn't simply count years of study, though: under the law it assesses whether your qualification is *equivalent to, or as satisfactory as*, an approved New Zealand qualification for your scope — looking at content, clinical hours and outcomes, not just the level alone.

Top tip

If you're not certain how your qualification compares, it's worth checking before you apply rather than after. A qualification that's close to but not clearly at the expected level may still succeed on equivalence — but it helps to know early so any gaps can be addressed up front.

Recent practice

What is required: evidence that you have practised recently and to a sufficient extent.

Why it exists: skills fade when they aren't used. Recent, regular practice gives the Board assurance that your competence is current — not just that you were competent at some point in the past. If you have had a significant break from practice, flag it early so it can be addressed rather than discovered mid-assessment.

English language

What is required: evidence of English language ability (covered in detail in Section 07).

Why it exists: clear communication is a patient-safety issue. Imaging and radiation professionals must understand referrals, explain procedures, and communicate precisely with patients and colleagues.

Language evidence protects everyone involved.

Fitness to practise

What is required: a declaration (and sometimes evidence) about your physical and mental health as it relates to safe practice.

Why it exists: this is about ensuring you can practise safely, not about excluding people with health conditions. The vast majority of declarations raise no issues at all.

Professional conduct

What is required: disclosure of any disciplinary history, conditions or investigations.

Why it exists: the Board must protect patients. Full, honest disclosure — even of matters you think are minor or resolved — is always the right approach. Non-disclosure is treated far more seriously than the underlying matter usually is.

Certificates of Good Standing

What is required: a Certificate of Good Standing (sometimes called a Letter of Good Standing or Certificate of Current Professional Status) from every regulator you have been registered with.

Why it exists: these certificates are how one regulator formally tells another that you are in good standing, with no unresolved concerns. They are issued **directly between regulators** and have a limited validity period — which makes their timing one of the trickier parts of an application. More on this in Section 05.

Top tip

Read every requirement as a question the Board is trying to answer about your safety to practise. When you frame your evidence as the *answer* to that question, your application becomes far more persuasive.

Preparing your application

This is the practical heart of the guide. A strong application is rarely about doing anything clever — it's about assembling clean, complete, correctly certified documents and presenting them well. Here's every document, and how to get each one right.

For each document below we explain **why it is required**, **where to obtain it**, the **common mistakes** we see, and a **practical tip** to keep things moving.

Present it like a professional portfolio

How your application *looks* matters more than people expect. The Board is forming a picture of you as a professional, and a folder of crooked, poorly photocopied, half-legible documents quietly undermines that picture — even when every document is genuine and correct. Treat your application like a portfolio you'd be proud to hand over, not a pile of paperwork.

In practice that means: clean, high-resolution colour scans; pages straight and square, not photographed at an angle; nothing cut off at the edges; consistent file naming; and certification and translations done properly the first time. A tidy, complete, well-ordered application is easier to assess, harder to query, and simply reads as more credible.

Passport

Why: confirms your identity and nationality, and underpins your visa and right to work.

Where: your existing passport — just make sure it is valid and won't expire mid-process.

Common mistake: submitting a passport close to expiry, or a low-resolution scan that crops the machine-readable strip at the bottom.

Tip: scan the full photo page flat, in colour, with all four edges visible.

Qualification certificates

Why: these prove you hold the qualification your registration is based on.

Where: your awarding university or college; request certified copies or replacements early if you can't find the originals.

Common mistake: providing only a final certificate when the Board also wants to see the detail of what you studied.

Tip: keep certificates and transcripts together — they are usually assessed as a pair.

Academic transcripts

Why: transcripts show course content, clinical hours and modules — the detail behind the certificate. They are often what allows the Board to map your training to a New Zealand scope.

Where: your institution's registry or records office. Many can issue official transcripts directly.

Common mistake: missing pages, or transcripts that don't clearly show clinical placement hours.

Tip: if your transcript is light on clinical detail, ask your institution for a course outline or syllabus to accompany it.

Employment history & CV

Why: your work history demonstrates recent practice and the breadth of your experience.

Where: compiled by you — but it must be accurate and complete.

Common mistake: unexplained gaps, or a CV whose dates don't match the rest of the application.

Tip: account for every period, including gaps. A one-line explanation of a career break is far better than an unexplained hole.

Certificates of Good Standing

Why: they confirm you are in good standing with every regulator you've registered with (see Section 04).

Where: requested from each regulator, and sent **directly to the MRTB** — not via you.

Common mistake: requesting them too early, so they expire before your application is assessed; or forgetting a regulator you registered with years ago in another country.

Tip: these have a limited validity window, so *timing is everything*. Line them up to arrive when your application is ready, not months before.

Common mistake

Certificates of Good Standing expire. Requesting them at the very start of a long preparation period is one of the most frequent causes of avoidable delay — they can lapse before assessment even begins, and then have to be requested all over again.

Employment references

Why: references verify your competence and recent practice. They are so important that the whole of Section 06 is devoted to them.

Where: from current and recent supervisors or managers.

Common mistake: choosing a referee based on seniority rather than on how well they know your clinical work.

Tip: brief your referees properly — turn to Section 06 before you approach anyone.

Name change documents

Why: if your name differs across documents — for example after marriage — the Board needs to link them to one person.

Where: marriage certificate, deed poll, or equivalent legal document.

Common mistake: overlooking a name difference between a degree certificate and a passport.

Tip: check every document carries a consistent name *before* you submit, and include the bridging document if it doesn't.

Certified copies & translations

Why: certification confirms a copy is a true likeness of the original; translation makes non-English documents assessable.

Where: certification by an approved person (such as a notary or other authorised professional); translation by a recognised, independent translator.

Common mistake: incorrect or incomplete certification wording, or translations that aren't independently certified.

Tip: confirm exactly *who* is permitted to certify and how the certification must be worded before you spend money — getting this wrong means doing it twice.

Top tip

Create a single folder — physical or digital — with one clearly named file per document. When everything is gathered, certified and in date *at the same time*, the application itself takes a fraction of the effort.

Employment references

If there is one part of your application worth investing real care in, it's your references. They are often the deciding evidence of your **current competence** — and the part most within your influence to get right.

Why references matter

Your qualification proves what you were trained to do. Your references prove what you **actually do, now, in clinical practice**. The Board relies on them to confirm that you practise safely, competently and to the standard expected. A vague or generic reference leaves a gap the Board may have to fill by asking for more — which means delay.

Choosing the right referee

The instinct is to ask the most senior person available. Resist it. The best referee is someone who has **directly supervised or worked alongside your clinical practice** recently and can speak to it in specific terms. A clinical lead who knows your work beats a department head who barely does.

What makes a strong clinical reference

- It is written by someone in a credible position to assess your practice.
- It describes your **actual clinical work** — modalities, settings, responsibilities.
- It comments on your competence, safety and professionalism with specific examples.
- It confirms the **dates and extent** of the practice it describes.
- It is on official letterhead, signed, dated, and includes the referee's role and contact details.

Top tip

A strong reference is *specific*. "A competent and reliable practitioner" says little. "Independently performs CT and MRI examinations across our department, including complex trauma cases, to a consistently high standard" tells the Board exactly what it needs to know.

What information should be included

A reference that contains all of the following rarely prompts a follow-up request:

- The referee's full name, job title and professional relationship to you.
- How long, and in what capacity, they have known your work.
- The specific scope and modalities you work within.
- An assessment of your clinical competence and safety.
- Confirmation of dates and the hours/extent of your practice.
- Official letterhead, signature, date and verifiable contact details.

Examples: good vs. poor

⚠️ A poor reference

"I have known Sam for some years. Sam is hard-working and a pleasure to work with. I am happy to recommend Sam."

No clinical detail, no dates, no scope, no basis for assessing competence.

✓ A strong reference

"As Imaging Lead, I have directly supervised Sam for three years. Sam independently performs general X-ray, CT and MRI to a high standard, including complex and emergency cases, and consistently demonstrates safe, patient-centred practice."

Specific, dated, scope-clear, credible.

Common mistakes

Common mistakes with references

Referees chosen for seniority rather than clinical knowledge; references that are warm but vague; missing letterhead or contact details; no confirmation of dates or hours; and leaving the request so late that a referee is on leave when you need them.

How to brief your referee

Most weak references are weak simply because the referee wasn't told what was needed. When you ask, give them context: explain that this is for professional registration in New Zealand, share which scope you're applying for, and tell them the specific points the Board values — clinical detail, dates, scope and a clear assessment of competence. A well-briefed referee writes a far stronger letter, usually with less back-and-forth.

How Ethicare can help

We can **review your references before submission** against what the Board looks for, and flag anything that's likely to prompt a follow-up — while it's still easy to address. We support and guide; the content always remains your referee's own honest, independent assessment.

SECTION 07

English language requirements

English language evidence exists for one reason: patient safety. Clear communication in a clinical setting is non-negotiable, so the Board needs objective assurance of your ability.

IELTS

The International English Language Testing System is one of the most widely accepted tests. For registration you will normally need the **Academic** version, and the Board sets minimum scores both overall and in each component (listening, reading, writing, speaking). A strong overall score won't help if one component falls below the minimum, so prepare evenly across all four.

OET

The Occupational English Test is a healthcare-specific alternative that many clinicians prefer, because its tasks are set in realistic medical contexts. Like IELTS, it has minimum grades required in each of the four components.

Alternative evidence

In some circumstances — for example where you trained and practised extensively in English — the Board may accept alternative evidence in place of a test, such as employer references or evidence of an English-medium education and career. Eligibility for this route is specific and decided by the Board, so check before assuming you qualify.

Validity periods

Common mistake

Language test results expire — typically around two years from the test date. Sitting your test too early, then taking many months to assemble the rest of your application, is a frequent cause of having to sit (and pay for) the test again. Time it to remain valid through to assessment.

Preparing for testing

Even fluent, native English speakers can stumble on test technique. The format, timing and marking criteria reward preparation. Sit a practice test under timed conditions first, identify your weakest component, and focus your effort there. Booking early also matters — test centre availability can be limited.

Frequently asked questions

Which test should I choose? Either is fine where accepted; many clinicians find OET more natural because the content is medical. Pick the one that plays to your strengths.

Can I combine scores from two sittings? The Board's rules on combining sittings are specific — check current guidance before relying on it.

What if I narrowly miss one component? You generally need to re-sit; build a little time buffer into your plan in case.

Completing the application

Once your documents are gathered, certified and in date, the application itself is largely a matter of careful assembly. Here's the process, step by step.

1

Create your application

Set up your application with the Board and work through each section methodically. Read every question carefully — accuracy here saves queries later.

2

Upload your documents

Attach each document in the format requested. Use clear, high-quality colour scans, named so the Board can immediately see what each file is.

3

Pay the fees

Settle the applicable assessment and registration fees. Keep your payment confirmation.

4

Submit

Review everything one final time against the requirements, then submit. A ten-minute final check is the cheapest delay-insurance there is.

5

Respond to any requests

If the Board asks for additional information, reply promptly and completely. This is normal — speed of response is the biggest factor within your control.

Typical communication during assessment

Expect to hear from the Board at a few points: an acknowledgement that your application has been received, possibly one or more requests for further information, and finally the decision. Keep an eye on your email — including spam folders — and respond quickly. A request that sits unanswered for three weeks adds three weeks to your timeline.

Top tip

Before you submit, put yourself in the assessor's shoes and ask: "Could a stranger open this application and find everything they need without emailing me?" If the answer is yes, you've prepared it well.

Understanding the fees

Registration involves fees, and it helps to understand what you're paying for rather than being surprised by a list of charges. Depending on your circumstances, these can include an **application / qualification assessment fee**, the **registration fee** itself, your **Annual Practising Certificate (APC)**, and — if it applies to you — an **examination fee**. On top of the Board's own charges there are usually third-party costs, such as criminal history checks and any document translation or certification you need.

A few practical points. The examination fee is paid *in advance* and is forfeited if you're refused access on the day, so don't pay it until you're ready and your ID is in order. And as part of an Ethicare package, **APC costs are reimbursed**, so that particular charge needn't fall on you.

Always check the current figures

We've deliberately not printed fee amounts here. The Board publishes its prescribed fees in a fees notice on its website, and these are reviewed and updated periodically — so any figure we quoted could quickly be out of date. Always confirm the current amounts on the Board's official fees schedule before you budget or pay. We're happy to point you to the right page.

After you submit

Submitting your application is a milestone, not the finish line. Here's what happens next, so there are no surprises.

Acknowledgement

The Board will confirm your application has been received. This is your cue to keep your documents and login details organised and to watch for any further contact.

Assessment

Your qualification, competence and recent practice are reviewed against the standard for your scope. Timeframes vary depending on your background and how complete your application is — which is exactly why the preparation in Sections 04–08 pays off.

Further information requests

You may be asked for additional documents or clarification. As we've stressed throughout: this is routine, and a fast, complete response keeps things moving.

Registration decision

Once the Board is satisfied, it confirms your registration — sometimes with conditions. This is the moment your New Zealand career becomes real.

Annual Practising Certificate

With registration confirmed, you obtain your **APC**, which permits you to actually practise. As part of an Ethicare package, APC costs are **reimbursed** — so this step doesn't fall on you financially.

Finding employment

With registration (or clear registrability) in hand, you're in a strong position to secure a role. This is where relocation and settlement begin — and where Ethicare's relocation support takes over from the registration journey.



Common mistakes to avoid

Almost every avoidable delay we see traces back to one of the issues below. Read this page as a pre-flight checklist — get these right and your application will move as smoothly as it can.

⚠ Incomplete references

Warm but vague references with no clinical detail, dates or scope. The single most common cause of a follow-up request.

⚠ Poor-quality scans

Cropped, blurry or greyscale copies where text or edges are unclear. If the assessor can't read it, it doesn't count.

⚠ Missing transcripts

Submitting a certificate without the transcript that shows course content and clinical hours behind it.

⚠ Incorrect certification

Copies certified by the wrong person or with the wrong wording — meaning the work has to be redone.

⚠ Expired Good Standing

Certificates of Good Standing requested too early, lapsing before assessment and needing to be reissued.

⚠ Inconsistent names

A name that differs across passport, certificates and references with no bridging document to link them.

⚠ Gaps in employment history

Unexplained periods in your work history that raise questions about recent practice.

✓ The fix for all of these

Prepare everything together, check it as a set, and have it reviewed before you submit. Most of these mistakes are invisible to the person making them — a second pair of eyes catches them.

Frequently asked questions

These are the questions internationally qualified professionals ask us most often.

Can I apply for registration before finding a job?

Yes — and in most cases you should. Because registration usually needs to come before employment, starting it early puts you in a far stronger position when roles arise.

Do I need registration before interviewing?

Not always for an initial conversation, but employers want confidence that you are registrable. Being registered, or clearly on track to be, makes you a much stronger candidate.

How long does registration usually take?

It varies considerably depending on your background, your scope and — crucially — how complete your application is. A clean, well-prepared application is the single biggest thing you can do to keep the timeline short.

Can I submit additional documents later?

Yes. If the Board requests further information, you provide it during assessment. Responding promptly and fully is what keeps things moving.

Can I work while my application is being assessed?

You cannot practise in a regulated role until you are registered and hold a current APC. Plan your move around that fact rather than assuming you can begin and sort registration out afterwards.

Will I have to work under supervision?

Quite possibly — and that's normal. Many internationally qualified practitioners are registered with an initial period of supervised practice while they settle into the New Zealand system. Those who trained in countries with very comparable systems (such as the UK, Ireland, Australia and Canada) often face little or none; others commonly complete one. It isn't a judgement on your ability, and the Board decides it case by case.

Will I need to sit an examination?

Some applicants do. The Board offers an online examination as a route to registration where it assesses your qualification as not straightforwardly equivalent to the New Zealand standard. It's more likely for Sonographers and MRI Technologists, because those scopes have no fast-tracked country pathway — even Australian-qualified applicants go through full overseas assessment. If it applies, it's a defined step rather than an obstacle, and we'll help you understand what to expect.

What if my manager (referee) has left the organisation?

This is common and manageable. A referee can still provide a reference after moving on, provided they knew your clinical work. Alternatively, another suitable supervisor who oversaw your practice can step in. The key is genuine knowledge of your work, not their current employer.

How much does registration cost?

There are several fees — typically an assessment fee, the registration fee, your APC, and an examination fee if one applies — plus third-party costs like criminal checks and translations. We've not quoted figures here because the Board updates them periodically; always check its current fees notice. As part of an Ethicare package, APC costs are reimbursed.

Can Ethicare review my application?

Yes. We can review your documents and references before submission against what the Board looks for, and flag likely issues while they're still easy to fix. We support and guide you through the process — the registration decision always rests with the Board.

Your registration journey

From first decision to first day at work — here is the whole journey on a single page. Keep it nearby; it's the map you'll return to throughout.

{journey_html}

HOW ETHICARE WILL SUPPORT YOU

You don't have to navigate this alone

If you choose to partner with Ethicare, we'll support you from first enquiry through registration to relocation and your first day at work — with a single point of contact, honest guidance, and help at every stage. We support and guide you through the process; the registration decision always rests with the Medical Radiation Technologists Board.

READY TO BEGIN?

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